

Look for and manage RR-TB treatment side effects

Report adverse events via the MedSafety App, the reporting website, or complete adverse reporting form and email to adr@sahpra.org.za or fax to (021) 448 6181 or (012) 842 7609/10.

Side effect		TB medication likely to cause side effect	Management : consult latest NDoH guideline or discuss with PCAC/NCAC.
Chest pain, palpitations		Bedaquiline, clofazimine, delamanid, moxifloxacin	Do ECG and discuss with PCAC/NCAC same day.
Faintness		Bedaquiline, clofazimine, delamanid, moxifloxacin	Do ECG and discuss with PCAC/NCAC same day.
Dizziness		Bedaquiline, clofazimine, delamanid, moxifloxacin, amikacin	- Do ECG and discuss with PCAC/NCAC same day. - If on amikacin, stop amikacin and present to PCAC/NCAC for medication substitution¹.
Jaundice		Most RR-TB medications	Stop all medications and refer same day.
Nausea, vomiting, abdominal pain		Most RR-TB medications	- Check ALT and review result within 24 hours: - If ALT $\geq$ 100, stop all medications and refer same day. - If ALT 50-99, doctor to assess for possible causes, consider interrupting treatment and repeat ALT within 1 week. If unsure, discuss with specialist. - If nausea/vomiting: - Reassure that this usually improves after a few weeks. - Advise to eat a non-fatty meal before taking medication. - If no better, give metoclopramide 10mg to take 30 minutes before taking RR-TB medication. - If still no better and on ethionamide, give ethionamide in divided doses.
Skin rash/itch		Most RR-TB medication	Assess and manage $\hookrightarrow$ 67.
Seizures		Terizidone, levofloxacin, high dose INH	Manage seizure $\hookrightarrow$ 19 and refer same day.
Psychosis		Terizidone, high dose INH, levofloxacin, ethionamide	Manage psychosis $\hookrightarrow$ 85 and discuss/refer same day.
Change in vision	Change in visual acuity	Linezolid, ethambutol	- Stop linezolid and ethambutol and refer to eye specialist same day. - Discuss possible medication substitution¹ with PCAC/NCAC.
	Painful/red eyes, blurred vision, sensitive to light	Rifabutin	Stop rifabutin and refer to eye specialist same day.
Hearing loss/ringing in ears		Amikacin	Stop amikacin and discuss possible medication substitution ¹ with PCAC/NCAC.
Diarrhoea		Ethionamide, PAS, delamanid, bedaquiline, linezolid	- Reassure that this usually improves and advise to increase fluid intake. - Assess further $\hookrightarrow$ 46. Give loperamide 4mg initially, then 2mg after each loose stool, up to 12mg/day. - If severe and not resolving, discuss with PCAC/NCAC. Consider taking blood for Na, K⁺ and creatinine.
Joint pain		Pyrazinamide, levofloxacin, delamanid, bedaquiline	- Give ibuprofen 400mg 8 hourly as needed with food for up to 5 days (avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease). - If available, refer for physiotherapy. - If no improvement, discuss possible withdrawal of pyrazinamide with PCAC/NCAC.
Pain/numbness of feet		Terizidone, high dose INH, linezolid	Peripheral neuropathy likely, discuss with PCAC/NCAC.
Headaches		Linezolid, delamanid, bedaquiline	- Give paracetamol 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days. - Also consider other cause of headache $\hookrightarrow$ 30.
Skin darkening		Clofazimine	Reassure that this will improve after treatment completed.
Low mood or anxiety		Terizidone, high dose isoniazid	Assess low mood or anxiety $\hookrightarrow$ 86. If antidepressant started, avoid amitriptyline (prolongs QT interval).
Dry skin		Clofazimine	- Wash with aqueous cream (UEA) instead of soap. Avoid using aqueous cream as moisturiser (emollient). - Moisturise skin with emulsifying ointment (UE) twice a day.

¹Continue other medications while awaiting response from PCAC/NCAC.